NAME: _____ CLASS: _____ TEACHER: _____

KEY TOPIC 5 • 6 ENQUIRY HOURS • 1914–1918

The British Sector of the Western Front, 1914–1918

Injuries, Treatment and the Trenches in the British Sector



Primary Visual Evidence: *Stretcher Bearers of the RAMC Lifting a Wounded Man out of a Trench* • Gilbert Rogers (c. 1919)

OFFICIAL EDEXCEL SOURCE ARCHIVE • IWM ART 2485 / RAMC HISTORIC COLLECTION

1. FEATURE DRILLS [4M]
Q1(a) & Q1(b): Two 2-mark questions. Identify 1 specific feature + 1 supporting factual detail (5 mins).

2. SOURCE UTILITY [8M]
Q2(a): 3-step evaluation of Content, Provenance (Nature, Origin, Motive), and Contextual Limitations (10 mins).

3. FOLLOW-UP ENQUIRY [4M]
Q2(b): 4-row tabular enquiry: Detail, Historical Question, Source Type, and Clinical Value (5 mins).

COURSE SPECIFICATION • KEY ENQUIRY SEQUENCE (1914–1918)	LEARNT	REVISED
Key Topic 5.1: The Theatre of War: The British Sector, Trench Geography & Battles *Why were trenches designed in zig-zag traverses, and how did terrain at Ypres, Somme, and Arras affect medical treatment?*	☐	☐
Key Topic 5.2: The Trench Environment: Mud, Vermin & Non-Combat Illnesses *How did frontline conditions cause trench foot, trench fever, and dysentery, and what preventive routines were enforced?*	☐	☐
Key Topic 5.3: Battlefield Trauma: High Explosive Shrapnel, Gas Attacks & Infection *Why did explosive artillery produce gas gangrene, and how did the Brodie steel helmet and respirators reduce fatalities?*	☐	☐
Key Topic 5.4: The Chain of Evacuation: Stretcher Bearers, RAP, Dressing Stations & CCS *How did the RAMC and FANY evacuate wounded soldiers through six stations, and why was triage critical at CCSs?*	☐	☐
Key Topic 5.5: Surgical Breakthroughs: The Thomas Splint, Wound Debridement & Mobile X-Rays *How did the Thomas Splint slash femur mortality from 80% to 20%, and why was Carrel-Dakin irrigation necessary?*	☐	☐
Key Topic 5.6: Lifesaving Innovations: Blood Storage, Brain Surgery & Plastic Reconstruction *How did Oswald Robertson establish the first blood bank at Cambrai, and how did Gillies rebuild shattered faces at Sidcup?*	☐	☐

LIVING TIMELINE • PART 1: FRONTLINE CONDITIONS & EARLY TRAUMA (1914–1915)

Instructions: As you study each enquiry lesson, complete the timeline missions by sketching and annotating in the corresponding Key Topic boxes below.

OCT 1914 • Flanders Mud & The 1st Battle of Ypres

Key Topic 5.1

The British Expeditionary Force (BEF) holds the Ypres Salient. Low-lying Flanders clay creates a dangerously high water table; trenches fill with liquid mud. Shelling smashes all natural drainage canals, leaving men standing waist-deep in water and giving rise to the first epidemics of trench foot and dysentery.

APR 1915 • 2nd Battle of Ypres: The First Poison Gas Attack

Key Topic 5.2 • 5.3

German forces release 160 tons of chlorine gas across a 4-mile front. British troops suffer suffocation and fluid in the lungs; soldiers improvise with urine-soaked pads before chemically impregnated cotton-pad respirators (the Hypo Helmet) are rushed to the front. Lice-borne trench fever breaks out across all divisions.

DEC 1915 • The Thomas Splint & Introduction of the Brodie Helmet

Key Topic 5.3 • 5.5

Robert Jones introduces the Thomas Splint to frontline medical posts, stabilizing compound fractures and reducing femur mortality from 80% to 20%. Simultaneously, the War Office begins issuing John Brodie's stamped steel helmet, reducing penetrating shrapnel head trauma by an estimated 75–80%.

If you think your walk to period 1 is rough, try carrying a 14-stone sergeant through 4 miles of knee-deep Flanders clay.

LIVING TIMELINE • PART 2: MASS OFFENSIVES & CLINICAL INNOVATION (1916–1917)

Instructions: Complete the timeline missions by sketching and annotating in the corresponding Key Topic boxes below.

JULY 1916 • The Battle of the Somme: The Triage Crisis

Key Topic 5.4

The BEF suffers 57,000 casualties on 1 July 1916. Stretcher bearers and Casualty Clearing Stations are overwhelmed; motor ambulances struggle along single rutted roads. As a direct result, the RAMC enforces a strict three-tier triage system (Walking Wounded, Immediate Surgery, Moribund) to prevent total collapse.

APR–NOV 1917 • Arras Underground Hospital & Passchendaele Mud

Key Topic 5.1 • 5.4

At Arras, New Zealand tunnelers excavate Thompson's Cave—a 700-bed underground hospital with operating theatres, running water, and electric lights 20m beneath the chalk. At 3rd Ypres (Passchendaele), artillery destroys all drainage systems, creating knee-deep liquid mud requiring 6 bearers per stretcher.

NOV 1917 • Oswald Robertson's Blood Depot & Gillies Facial Reconstruction

Key Topic 5.6

At the Battle of Cambrai, Captain Oswald Robertson deploys the world's first blood bank, using sodium citrate to stop clotting, glucose to preserve red cells, and ice chests to store Group O blood for 28 days. In Britain, Harold Gillies pioneers plastic facial surgery using tubed pedicles at Queen's Hospital, Sidcup.

How did the physical terrain and trench architecture of the British sector dictate medical evacuation and care?

AUDIO • QUIZ

1 1914–1915 • Why was the low-lying topography of Ypres a tactical and medical nightmare?

• Ypres Salient waterlogged	
• Clay soil drainage destruction	
• Deep mud stretcher crisis	

2 1914–1918 • How did zig-zag traverses limit shrapnel injuries and contain shell explosions?

• Zig-zag traverse construction	
• Duckboards & sump drainage	
• Frontline firing step	

3 1915–1918 • Why did narrow communication trenches create acute bottlenecks for stretchers?

• Three-line trench system	
• Communication trench transit	
• Narrow stretcher passages	

4 Nov 1916–1917 • How did the chalk geology at Arras allow safe, sterile surgical conditions?

• Arras chalk caverns (1916)	
• Thompson's Cave hospital	
• 700 underground beds	

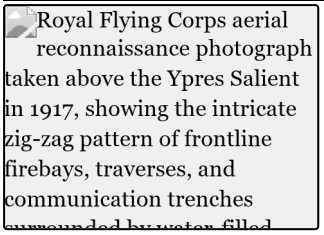
5 1916–1917 • Why did mass-casualty offensives force the RAMC to reorganize evacuation?

• 1916 Somme casualty flood	
• 57,000 first-day casualties	
• 1917 Cambrai tank battle	

Welcome to Flanders: 500 square miles of fermented pig manure, liquid mud, and artillery with terrifying accuracy.

Question 2(a): Source Utility Assessment [8 marks • 10 mins]

SOURCE A • CONTEMPORARY AERIAL RECONNAISSANCE • ROYAL FLYING CORPS **ORIGIN: 1917**


Royal Flying Corps aerial reconnaissance photograph taken above the Ypres Salient in 1917, showing the intricate zig-zag pattern of frontline firebays, traverses, and communication trenches surrounded by water-filled

Royal Flying Corps aerial reconnaissance photograph taken above the Ypres Salient in 1917, showing the intricate zig-zag pattern of frontline firebays, traverses, and communication trenches surrounded by water-filled shell craters.

Historical Provenance: Imperial War Museum / Contemporary Photographic Record (1914–1918).

Study Source A. How useful is Source A for an enquiry into the defensive layout and tactical design of trench systems on the Western Front? Explain your answer, using Source A and your knowledge of the historical context. [8 marks]

Provenance Clue: Consider the nature of an official military aerial reconnaissance photograph taken by the Royal Flying Corps in 1917. What does an overhead view reveal about traverses, fire-steps, and saps, and what ground-level medical and mud conditions does it omit?

ANALYTICAL CONNECTIVES: *Source A is useful for an enquiry into this topic because... • Specifically, the source reveals... • In terms of provenance, considering the author and date... • However, the utility is limited because... • Therefore...*

Write your response to Question 2(a) in the space provided below:

(Response continues on facing page →)

TIMELINE MISSION Turn to Pages 2–3 (Milestone 1). Sketch and annotate the zig-zag traverse pattern and low-lying Ypres water table. **[ ] Done**

Continue your response to Question 2(a) below:

(Concludes on next page →)

Final analytical evaluation & concluding historical verdict for Question 2(a):

• EDEXCEL SECTION A: QUESTION 1 FEATURE DRILLS [4 MARKS • 5 MINS] **2 SEPARATE QUESTIONS • 2 MARKS EACH**

Examiner Mark Scheme: Award 1 mark for identifying a valid historical feature + 1 mark for supporting factual detail. Supporting detail must directly develop the named feature.

1(a) Describe one feature of the system of communication trenches on the Western Front. **[/2]**

FEATURE:

DETAIL:

1(b) Describe one feature of the underground hospital at Arras (Thompson's Cave). **[/2]**

FEATURE:

DETAIL:

Why were environmental pathogens and vermin as dangerous to British troops as enemy artillery?

AUDIO • QUIZ

1 1914–1915 • How did prolonged cold water immersion destroy tissue and produce gangrene?

• Trench foot fungal infection	
• Constricted circulation numbness	
• Amputation gangrene risk	

2 1915–1918 • Why did mandatory officer inspections succeed where voluntary advice had failed?

• Whale oil rubbing	
• Dry sock changes	
• Daily officer foot inspections	

3 1914–1918 • Why did trench fever persist until the body louse insect vector was proven in 1918?

• Trench fever pyrexia	
• Body lice transmission	
• Delousing bathhouses	

4 1914–1918 • How did strict sanitation and chemical water chlorination prevent epidemics?

• Bacterial dysentery illness	
• Latrine chloride-of-lime	
• Purified water chlorination	

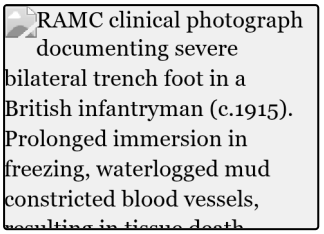
5 1914–1918 • Why was shell shock initially stigmatised as cowardice before being treated medically?

• Shell shock (NYDN)	
• Artillery concussive trauma	
• Craiglockhart mental convalescence	

Whale oil smells like dead fish and regrets, but it beats having your toes amputated by an RAMC surgeon with a bone-saw.

Question 2(b): Follow-Up Enquiry Table [4 marks • 5 mins]

SOURCE A • CONTEMPORARY PHOTOGRAPHIC EVIDENCE • WESTERN FRONT **ORIGIN: July 1916**


RAMC clinical photograph documenting severe bilateral trench foot in a British infantryman (c.1915). Prolonged immersion in freezing, waterlogged mud constricted blood vessels, resulting in tissue death

RAMC clinical photograph documenting severe bilateral trench foot in a British infantryman (c.1915). Prolonged immersion in freezing, waterlogged mud constricted blood vessels, resulting in tissue death, blistering, and wet gangrene.

Historical Provenance: Imperial War Museum / Contemporary Photographic Record (1914–1918).

Study Source A. How could you follow up Source A to find out more about the living conditions and non-combat illnesses experienced by British soldiers in frontline trenches on the Western Front? [4 marks]

Examiner Strategy: Follow up an exact quotation or visual detail. Your question must directly target your chosen detail. Your named source type must be authentic and specific (e.g. RAMC casualty returns, battalion war diaries).

ANALYTICAL CONNECTIVES: *A key detail to follow up is... • The question I would ask is... • The specific historical source type needed is... • This source would help answer my question because...*

Complete the 4-prompt historical enquiry matrix below: *(Response continues on facing page →)*

Detail in Source A that I would follow up:	
Question I would ask:	
Type of source I would use:	
How this would help me find out more:	

Disciplinary Commentary • Clinical Justification of Chosen Source Type:

TIMELINE MISSION Turn to Pages 2–3 (Milestone 2). Draw soldiers applying whale oil and delousing garments in steam ovens. **[ ] Done**

Continue your response to Question 2(b) below:

(Concludes on next page →)

Final analytical evaluation & concluding historical verdict for Question 2(b):

• **EDEXCEL SECTION A: QUESTION 1 FEATURE DRILLS [4 MARKS • 5 MINS]** **2 SEPARATE QUESTIONS • 2 MARKS EACH**

Examiner Mark Scheme: Award 1 mark for identifying a valid historical feature + 1 mark for supporting factual detail. Supporting detail must directly develop the named feature.

1(a) Describe one feature of the methods used to prevent trench foot on the Western Front. **[/2]**

FEATURE:

DETAIL:

1(b) Describe one feature of the causes of trench fever among British troops. **[/2]**

FEATURE:

DETAIL:

How did high-explosive artillery trauma and chemical poison gas transform surgical treatment?

AUDIO • QUIZ

1 1914–1918 • Why did jagged shrapnel fragments cause far worse infections than rifle bullets?

- Artillery shrapnel (58%)
- High explosive fragments
- Embedded uniform cloth

2 1914–1918 • Why was Clostridium welchii so lethal in deep, airless shrapnel wounds?

- Manured agricultural soil
- Clostridium gas gangrene
- Dead tissue bacteria

3 Late 1915 • Why did the introduction of the Brodie helmet cause an apparent rise in hospital head cases?

- Brodie steel helmet (1915)
- Shrapnel deflection rim
- Fatal head wounds halved

4 Apr 1915–1916 • How did the rapid evolution of chemical respirators counter suffocating gases?

- Chlorine gas Ypres (1915)
- Phosgene suffocation gas
- Urine-soaked pad protection

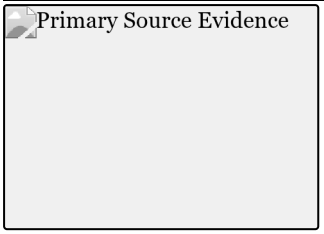
5 1916–1918 • Why was mustard gas far harder to counter than chlorine or phosgene?

- Mustard blistering agent (1917)
- Persistent chemical soil vapour
- Small Box Respirator

The Brodie helmet: Looks like an upside-down soup bowl, but prevents your skull from becoming one.

Question 2(a): Source Utility Assessment [8 marks • 10 mins]

SOURCE B • CONTEMPORARY PHOTOGRAPHIC EVIDENCE • MACHINE GUN CORPS **ORIGIN: July 1916**



Official photographic and archival evidence documenting medical facilities and conditions in the British sector of the Western Front.

Historical Provenance: Imperial War Museum / Contemporary Photographic Record (1914–1918).

Study Source B. How useful is Source B for an enquiry into the methods used to protect British soldiers from poison gas attacks on the Western Front? Explain your answer, using Source B and your knowledge of the historical context. [8 marks]

Provenance Clue: Consider the nature of an authentic British War Office Phenate-Hexamine (PH) anti-gas helmet manufactured in 1915–1916. What physical protective properties does chemically impregnated flannel possess, and what severe combat limitations did it have in battle?

ANALYTICAL CONNECTIVES: *Source B is useful for investigating this topic because... • Specifically, the source provides evidence that... • In terms of provenance, considering the nature and purpose... • However, the source is limited because... • Consequently...*

Write your response to Question 2(a) in the space provided below: *(Response continues on facing page →)*

TIMELINE MISSION Turn to Pages 2–3 (Milestone 2). Sketch the Brodie steel helmet and British PH gas hood. **[ ] Done**

Continue your response to Question 2(a) below:

(Concludes on next page →)

Final analytical evaluation & concluding historical verdict for Question 2(a):

• **EDEXCEL SECTION A: QUESTION 1 FEATURE DRILLS [4 MARKS • 5 MINS]** **2 SEPARATE QUESTIONS • 2 MARKS EACH**

Examiner Mark Scheme: Award 1 mark for identifying a valid historical feature + 1 mark for supporting factual detail. Supporting detail must directly develop the named feature.

1(a) Describe one feature of the effects of poison gas attacks on soldiers on the Western Front. **[/2]**

FEATURE:

DETAIL:

1(b) Describe one feature of the design of the British Brodie steel helmet. **[/2]**

FEATURE:

DETAIL:

How successfully did the Chain of Evacuation transport, triage, and treat mass battlefield casualties?

AUDIO • QUIZ

1 1914–1918 • Why was rapid evacuation by stretcher bearers vital to prevent irreversible shock?

• Four-man stretcher teams	
• Regimental Aid Post (RAP)	
• First aid & tourniquets	

2 1914–1918 • What was the tactical role of Dressing Stations in preventing CCS congestion?

• Advanced Dressing Stations (ADS)	
• Field Ambulance RAMC	
• Tetanus anti-toxin serum	

3 1914–1918 • How did the 3-tier triage system maximize survival during mass offensives?

• Casualty Clearing Station (CCS)	
• Three-tier triage sorting	
• Immediate life-saving surgery	

4 1914–1918 • Why were Base Hospitals situated near major coastal rail terminals and ports?

• Coastal Base Hospitals	
• Boulogne & Étaples centres	
• Specialised surgery & convalescence	

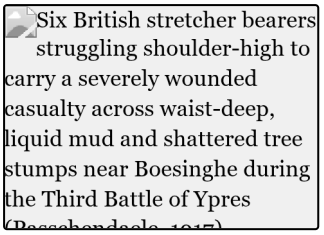
5 1914–1918 • Why were canal barges preferred over trains for head and chest trauma?

• Horse-drawn ambulance delays	
• Motor ambulances introduced	
• Canal barges head-trauma	

The Regimental Aid Post: 200 yards from the German front line, lit by candle-ends, and smelling entirely of iodine and panic.

Question 2(b): Follow-Up Enquiry Table [4 marks • 5 mins]

SOURCE A • CONTEMPORARY PHOTOGRAPHIC EVIDENCE • WESTERN FRONT **ORIGIN: August 1917**



Six British stretcher bearers struggling shoulder-high to carry a severely wounded casualty across waist-deep, liquid mud and shattered tree stumps near Boesinghe during the Third Battle of Ypres (Passchendaele, 1917).

Historical Provenance: Imperial War Museum / Contemporary Photographic Record (1914–1918).

Study Source A. How could you follow up Source A to find out more about the difficulties stretcher bearers faced evacuating casualties from the Western Front? [4 marks]

Examiner Strategy: Follow up an exact quotation or visual detail. Your question must directly target your chosen detail. Your named source type must be authentic and specific (e.g. RAMC casualty returns, battalion war diaries).

ANALYTICAL CONNECTIVES: *A key detail to follow up is... • The question I would ask is... • The specific historical source type needed is... • This source would help answer my question because...*

Complete the 4-prompt historical enquiry matrix below: *(Response continues on facing page →)*

Detail in Source A that I would follow up:	
Question I would ask:	
Type of source I would use:	
How this would help me find out more:	

Disciplinary Commentary • Clinical Justification of Chosen Source Type:

TIMELINE MISSION Turn to Pages 2–3 (Milestone 5). Draw the 6 sequential stages of the Chain of Evacuation. **[ ] Done**

Continue your response to Question 2(b) below:

(Concludes on next page →)

Final analytical evaluation & concluding historical verdict for Question 2(b):

• EDEXCEL SECTION A: QUESTION 1 FEATURE DRILLS [4 MARKS • 5 MINS] **2 SEPARATE QUESTIONS • 2 MARKS EACH**

Examiner Mark Scheme: Award 1 mark for identifying a valid historical feature + 1 mark for supporting factual detail. Supporting detail must directly develop the named feature.

1(a) Describe one feature of the triage system used at Casualty Clearing Stations on the Western Front. **[/2]**

FEATURE:

DETAIL:

1(b) Describe one feature of the work of the First Aid Nursing Yeomanry (FANY) on the Western Front. **[/2]**

FEATURE:

DETAIL:

Why were the Thomas Splint, debridement, and mobile X-rays decisive surgical breakthroughs?

AUDIO • QUIZ

1 1914–1918 • Why was surgical excision of dead tissue more effective than chemical antiseptics?

• Wound surgical debridement	
• Dead tissue excision	
• Delayed primary closure	

2 1915–1918 • How did continuous chemical irrigation solve the problem of deep wound infection?

• Carrel-Dakin antiseptic irrigation	
• Sodium hypochlorite bleach	
• Rubber perfusion tubes	

3 Dec 1915–1916 • How did rigid traction prevent fatal haemorrhage and shock from broken femurs?

• Thomas splint (1915)	
• Compound femur traction	
• Mortality reduced (80% to 20%)	

4 1914–1918 • Why was locating shrapnel on an X-ray vital before operating on gas gangrene?

• Mobile radiographic vans	
• Van dynamo generators	
• Locating embedded shrapnel	

5 1914–1918 • Why did undetected cloth fragments remain the primary cause of post-op gangrene?

• Fragile vacuum tubes	
• Overheating radiation limits	
• Undetected cloth fragments	

The Thomas Splint: Before Robert Jones introduced it, an 80% chance of death; after Jones, an 80% chance of living to complain about the food.

Question 2(a): Source Utility Assessment [8 marks • 10 mins]

SOURCE A • CONTEMPORARY MEDICAL ARTIFACT • STANDARD ISSUE **ORIGIN: c.1916**



Operating theatre suite inside No. 23 Casualty Clearing Station on the Western Front. Multiple operating tables with surgical instruments and overhead illumination enabled specialist surgical teams to perform urgent abdominal laparotomies and debridement simultaneously.

Historical Provenance: Imperial War Museum / Contemporary Photographic Record (1914–1918).

Study Source A. How useful is Source A for an enquiry into the methods used to treat wounded soldiers with fractured limbs on the Western Front? Explain your answer, using Source A and your knowledge of the historical context. [8 marks]

Provenance Clue: Consider the nature of an authentic instructional photograph taken in an RAMC military hospital training ward c.1916. What does it demonstrate about mechanical traction and splinting, and why might an instructional training photograph differ from emergency frontline conditions in a mud dugout?

ANALYTICAL CONNECTIVES: *Source A is useful for an enquiry into this topic because... • Specifically, the source demonstrates that... • In terms of provenance, considering the contemporary context... • However, the utility is limited because... • Therefore...*

Write your response to Question 2(a) in the space provided below: *(Response continues on facing page →)*

TIMELINE MISSION Turn to Pages 2–3 (Milestone 3). Sketch the Thomas Splint groin ring and traction cord. **[ ] Done**

Continue your response to Question 2(a) below:

(Concludes on next page →)

Final analytical evaluation & concluding historical verdict for Question 2(a):

• EDEXCEL SECTION A: QUESTION 1 FEATURE DRILLS [4 MARKS • 5 MINS] **2 SEPARATE QUESTIONS • 2 MARKS EACH**

Examiner Mark Scheme: Award 1 mark for identifying a valid historical feature + 1 mark for supporting factual detail. Supporting detail must directly develop the named feature.

1(a) Describe one feature of the Thomas Splint used on the Western Front. **[/2]**

FEATURE: _____

DETAIL: _____

1(b) Describe one feature of the Carrel-Dakin method of treating infected wounds. **[/2]**

FEATURE: _____

DETAIL: _____

How did stored blood banks, specialized neurosurgery, and plastic reconstruction transform wartime survival?

AUDIO • QUIZ

1 1914–1915 • How did sodium citrate liberate blood transfusion from donor-to-patient tubing?

- Richard Lewisohn sodium citrate
- Uncoupled indirect transfusion
- Preventing blood clotting

2 1916–1917 • Why was glucose addition essential to prevent red blood cell breakdown?

- Rous & Turner citrate-glucose
- Stored refrigerated blood
- Four-week viability window

3 Nov 1917 • Why did pre-stored blood transform surgical survival during surprise assaults?

- Oswald Robertson (1917)
- Cambrai blood depot
- Type O universal donors

4 1917–1918 • Why was local anaesthesia superior to general anaesthetic in head trauma?

- Harvey Cushing neurosurgery
- Local anaesthetic brain operations
- Magnet shrapnel extraction

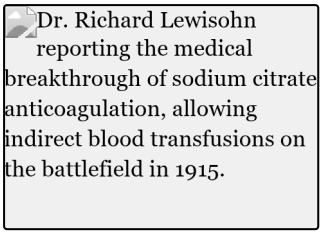
5 1917–1918 • How did the tubed pedicle technique prevent skin grafts from rotting and dying?

- Harold Gillies Sidcup (1917)
- Queen's Hospital facial surgery
- Tubed pedicle skin grafting

Robertson's Blood Depot: Ice chests, sodium citrate, and refrigerated blood at Cambrai—proof that cold beer isn't the only thing worth chilling.

Question 2(b): Follow-Up Enquiry Table [4 marks • 5 mins]

SOURCE A • PRIMARY MEDICAL ARTIFACT • CAMBRAI BLOOD DEPOT **ORIGIN: 1917**



Dr. Richard Lewisohn reporting the medical breakthrough of sodium citrate anticoagulation, allowing indirect blood transfusions on the battlefield in 1915.

Dr. Richard Lewisohn reporting the medical breakthrough of sodium citrate anticoagulation, allowing indirect blood transfusions on the battlefield in 1915.

Historical Provenance: Annals of Surgery, Vol. 62, New York, July 1915.

Study Source A. How could you follow up Source A to find out more about the success of stored blood transfusions at the Battle of Cambrai in 1917? [4 marks]

Examiner Strategy: Follow up an exact quotation or visual detail. Your question must directly target your chosen detail. Your named source type must be authentic and specific (e.g. RAMC casualty returns, battalion war diaries).

ANALYTICAL CONNECTIVES: *A key detail to follow up is... • The question I would ask is... • The specific historical source type needed is... • This source would help answer my question because...*

Complete the 4-prompt historical enquiry matrix below: *(Response continues on facing page →)*

Detail in Source A that I would follow up:	
Question I would ask:	
Type of source I would use:	
How this would help me find out more:	

Disciplinary Commentary • Clinical Justification of Chosen Source Type:

TIMELINE MISSION Turn to Pages 2–3 (Milestone 6). Sketch Oswald Robertson's iced blood chest and Harold Gillies' tubed pedicle. **[ ] Done**

Continue your response to Question 2(b) below:

(Concludes on next page →)

Final analytical evaluation & concluding historical verdict for Question 2(b):

• EDEXCEL SECTION A: QUESTION 1 FEATURE DRILLS [4 MARKS • 5 MINS] **2 SEPARATE QUESTIONS • 2 MARKS EACH**

Examiner Mark Scheme: Award 1 mark for identifying a valid historical feature + 1 mark for supporting factual detail. Supporting detail must directly develop the named feature.

1(a) Describe one feature of Harvey Cushing's surgical techniques in operating on head wounds on the Western Front. **[/2]**

FEATURE:

DETAIL:

1(b) Describe one feature of the surgical methods used by Harold Gillies at Queen's Hospital, Sidcup. **[/2]**

FEATURE:

DETAIL:

STUDENT ASSESSMENT RECORD • KEY TOPIC 5 TRACKER

The British Sector of the Western Front, 1914–1918: Injuries, Treatment and the Trenches

PUPIL: _____ **TARGET GRADE:** ☐ **PREDICTED:** ☐ **ATTITUDE:** 1 • 2 • 3 • 4 • 5

#	ENQUIRY / HISTORIC ENVIRONMENT FOCUS	DO NOW (10M)	Q1 FEATURES (4M)	Q2 SOURCE ENQUIRY	LESSON TOTAL
1	**KT5.1: The Theatre of War: The British Sector, Trench Geography & Battles** *"How did the physical terrain and trench architecture of the British sector dictate medical evacuation and care?"*	/ 10	/ 4	Q2(a) Utility: / 8	/ 22
2	**KT5.2: The Trench Environment: Mud, Vermin & Non-Combat Illnesses** *"Why were environmental pathogens and vermin as dangerous to British troops as enemy artillery?"*	/ 10	/ 4	Q2(b) Follow-up: / 4	/ 18
3	**KT5.3: Battlefield Trauma: High Explosive Shrapnel, Gas Attacks & Infection** *"How did high-explosive artillery trauma and chemical poison gas transform surgical treatment?"*	/ 10	/ 4	Q2(a) Utility: / 8	/ 22
4	**KT5.4: The Chain of Evacuation: Stretcher Bearers, RAP, Dressing Stations & CCS** *"How successfully did the Chain of Evacuation transport, triage, and treat mass battlefield casualties?"*	/ 10	/ 4	Q2(b) Follow-up: / 4	/ 18
5	**KT5.5: Surgical Breakthroughs: The Thomas Splint, Wound Debridement & Mobile X-Rays** *"Why were the Thomas Splint, debridement, and mobile X-rays decisive surgical breakthroughs?"*	/ 10	/ 4	Q2(a) Utility: / 8	/ 22
6	**KT5.6: Lifesaving Innovations: Blood Storage, Brain Surgery & Plastic Reconstruction** *"How did stored blood banks, specialized neurosurgery, and plastic reconstruction transform wartime survival?"*	/ 10	/ 4	Q2(b) Follow-up: / 4	/ 18
KEY TOPIC 5 CUMULATIVE ASSESSMENT TOTALS		**Do Now: / 60**	**Q1 Feat: / 24**	**Q2 Enq: / 36**	**/ 120**

TEACHER FORMATIVE ASSESSMENT • WWW / EBI FEEDBACK

WHAT WENT WELL (WWW):

_____

_____

_____

EVEN BETTER IF (EBI):

_____

_____

_____

Teacher Signature: _____ Date: _____

DIGITAL QUIZZING HUB • 20-QUESTION PRACTICE QUIZZES

Scan with smartphone camera to open live interactive 20-question self-marking quizzes

KT 5.1	KT 5.2	KT 5.3	KT 5.4	KT 5.5	KT 5.6
SCAN TO QUIZ	SCAN TO QUIZ	SCAN TO QUIZ	SCAN TO QUIZ	SCAN TO QUIZ	SCAN TO QUIZ
Score: / 20	**Score: / 20**	**Score: / 20**	**Score: / 20**	**Score: / 20**	**Score: / 20**

Retain this student assessment record for parent consultations and GCSE revision monitoring.